

The HTR System — Book + Platform Combined Deck

Purpose: Show how the book *Transforming American Healthcare* and the HTR Platform form **one system** — and how a reader/user works with both to actually master the concepts.

Audience: Serious readers, course adopters, institutional partners, advisory clients, anyone deciding whether to commit time to HTR.

Length: ~65 slides. --- per slide. Blockquoted speaker notes.

Slide 1 — Title

The HTR System

One framework. One book. One platform. Six pillars.

How to work with the book and the platform together to understand — and execute — healthcare transformation.

Open: HTR is built as a **system**, not a website or a book. This deck is the user manual for the system.

Slide 2 — The thesis (again, simply)

American healthcare is structurally failing. Reform must happen across **six pillars together**: Policy, Economics, Technology, Clinical, Equity, Operations.

The HTR system gives you:

- A **book** — the framework, evidence, and practitioner guidance
- A **platform** — the interactive tools, data, and live state intelligence

Used together, they let you do the work.

Slide 3 — Why one without the other doesn't work

Book without platform:

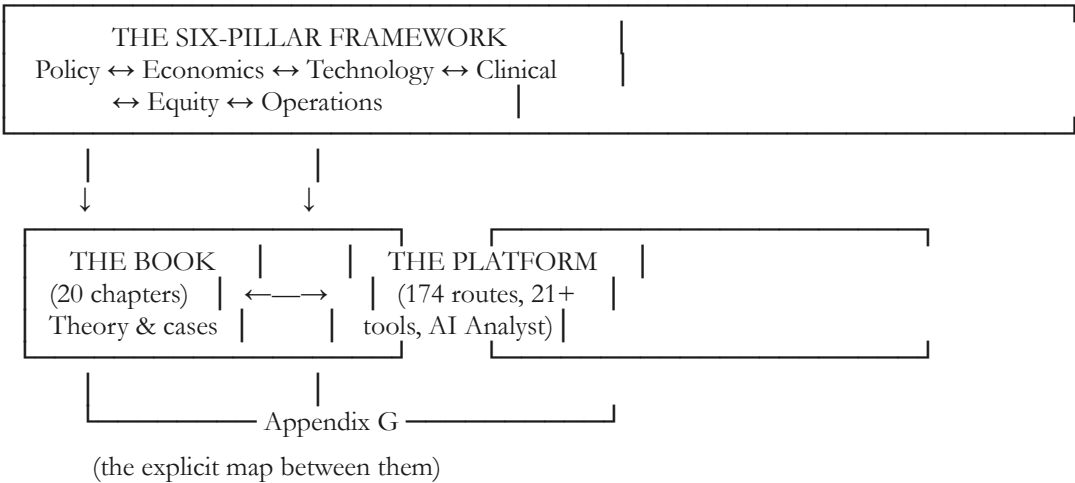
- You learn the framework, but cannot test it on real data
- You cannot run risk-stratification scenarios, simulate APMs, score your organization
- You lose currency — health policy changes monthly

Platform without book:

- You use the tools without understanding why they're structured the way they are
- You miss the dependency relationships between pillars
- You misread Vermont as "a small state with a big appetite" rather than as the structurally complete case

Together, they form a complete loop.

Slide 4 — The dual structure at a glance



Slide 5 — Three ways to use the system

1. **Read-first** — start with the book, use the platform to test what you read
2. **Tool-first** — start with the simulators, use the book to understand the results
3. **Question-first** — start with the AI Analyst, use both as the corpus

None is wrong. Most readers oscillate.

Slide 6 — Meet the four reader profiles

Profile	Primary frame
 Policy professional	Legislative architecture and political sustainability
 Healthcare executive	Operations, finance, and risk
 Vermont practitioner	Daily operational reality on the ground
 Student / researcher	Framework, methodology, and evidence base

Both the book (Appendix G) and the platform ([/book](#) page) recommend a tailored entry path for each profile.

Slide 7 — The Six Pillars (review)

Pillar	Color	Book chapters	Platform pillar page
Policy	Sky	4–5	/policy
Economics	Emerald	8–9	/economics
Technology	Indigo	6–7	/technology
Clinical	Red	10–11	/clinical
Equity	Violet	12–13	/equity
Operations	Teal	14–15	/operations

The platform's pillar accent colors are deliberate — they match the book's section dividers.

Slide 8 — The 15 dependencies (recap)

Six pillars, fifteen pairwise dependencies ($6 \times 5 \div 2$).

Book: Chapter 1 + Appendix B. Platform: [/about/framework](#) — an interactive map.

Both the book chapter and the platform map should be reviewed before doing any pillar-specific work.

Slide 9 — How to read Chapter 1

1. Read the chapter (book, ~25 pages)
2. Open [/about/framework](#) on the platform
3. Click each pillar — see the dependency graph live
4. Open the **HTR Simulator** ([/htr-simulator](#))
5. Score an organization (your own, or Vermont) — feel the dependencies operate

Chapter 1 + the Simulator together replaces a semester of theoretical health policy in one afternoon.

Slide 10 — How to read Chapter 2 (Sequencing)

1. Read the OneCare Vermont autopsy in the book
2. Open [/vermont-vcci](#) and [/ahead-model](#) on the platform
3. Re-read the chapter with VCCI's actual mechanics in mind
4. Use the **Impact Simulation** ([/impact-simulation](#)) to model an out-of-order sequence — watch the failure cascade

The chapter argues sequencing is causal, not preferential. The simulation makes that argument tactile.

Slide 11 — How to read Chapter 4 (Policy Pillar)

Book chapter covers Vermont Acts 167 & 68, the Oliver Wyman blueprint, global budget architecture, reference-based pricing.

Platform companion (Policy):

- [/vermont-act-167](#) — full Act 167 page + the Oliver Wyman report
- [/vermont-act-68](#) — the operational mandate
- [/vermont-rht-program](#) — the \$195M Rural Health Transformation Program
- Research Lab: **Policy Simulator** ([/research-lab/policy-quality?tab=policy](#))

Read the chapter; then run a policy scenario through the simulator. Pick a real bill — the analysis transfers directly.

Slide 12 — How to read Chapter 5 (Policy in Practice)

Book covers CMMI models, 1115 waivers, H.R. 1 implications.

Platform companion:

- [/research-lab/policy-quality?tab=hr1-cliff](#) — H.R. 1 Cliff Scenario
- [/research-lab/policy-quality?tab=medicaid-wr](#) — Work Requirements Calculator
- [/vermont-medicaid](#) — the Vermont-specific Medicaid page

Run the H.R. 1 cliff scenario on your state's eligibility numbers — the chapter argument becomes a budget memo.

Slide 13 — How to read Chapter 6 (Technology Pillar)

Book covers VHCURES, VITL, Act 62 governance, FHIR, the Vermont CIN, AI governance.

Platform companion:

- [/technology](#) overview
- [/technology/security](#) — data governance
- Research Lab: **FHIR Interoperability Lab**, **AI Clinical Governance Lab**

Chapter 6 is dense — the FHIR Lab makes the abstract interoperability stack concrete.

Slide 14 — How to read Chapter 7 (Technology in Practice)

Book covers AI scribe, RPM, telehealth, diagnostic AI, the AI Clinical Governance Lifecycle, alert fatigue.

Platform companion:

- [/research-lab/technology-ai?tab=ai](#) — AI Clinical Governance Lab
- [/research-lab/technology-ai?tab=digital](#) — Digital Health Lab
- [/research-lab/vbc-clinical-quality?tab=h17](#) — Clinical Data Exchange Lab

Use the AI Governance Lab to score an AI vendor against the lifecycle. The book's abstract framework becomes a procurement scorecard.

Slide 15 — How to read Chapter 8 (Economics Pillar)

Book covers the fee-for-service trap, Vermont's four global-budget attempts, Maryland's evidence, AHEAD integration.

Platform companion:

- [/ahead-model](#) — full AHEAD page
- [/vermont-act-68](#) — the legislative mandate
- Research Lab: **APM Design Lab**, **Global Budget Transition Modeler**

The Global Budget Modeler is the book chapter's primary tool. Set up a Vermont hospital under Act 68 and watch the revenue regime shift.

Slide 16 — How to read Chapter 9 (Economics in Practice)

Book has the **65-item VBC Contract Review Checklist** (Appendix D) — most-cited operational tool in the book.

Platform companion:

- Research Lab: **Shared Savings Calculator**, **CEA Calculator**
- **VBC Readiness Assessment** ([/research-lab/knowledge-workspace?tab=readiness](#))
- **Hospital Financial Stress Test**

Run the VBC Readiness Assessment for your organization; then walk through Appendix D with your legal and finance teams.

Slide 17 — How to read Chapter 10 (Clinical Pillar)

Book covers Blueprint, VCCI, SASH, DAs, behavioral health architecture, Collaborative Care, PACE.

Platform companion — exceptional density here:

- [/vermont-blueprint](#) · [/vermont-vcci](#) · [/vermont-sash](#) · [/vermont-designated-agencies](#)
- [/clinical](#) overview
- Research Lab: **Risk Stratification Engine** + **Risk Stratification Methodology** (VCCI-aligned)
- **VCCI Risk Stratification Lab** — interactive patient scenarios

Chapter 10 + the VCCI Lab + the VCCI page is the most integrated book-platform pairing in the entire system.

Slide 18 — How to read Chapter 11 (Clinical in Practice)

Book covers PCMH transformation, Blueprint field staff, HEDIS, Collaborative Care deployment.

Platform companion:

- Research Lab: **VBC Quality Measures, High vs. Low Value Care, Clinical Quality Optimizer**
- [/vermont-blueprint](#) deep dive

Use the VBC Quality Measures lab to walk through a HEDIS rate; pair with the chapter's HEDIS strategy.

Slide 19 — How to read Chapter 12 (Equity Pillar)

Book covers SDOH as structural, Vermont's 8 SDOH domains, algorithmic bias, access disparity.

Platform companion:

- [/vermont-sdoh](#) — Vermont's 8 SDOH domains operationalized
- [/equity/sdoh](#) · [/equity/bias](#) · [/equity/access](#)
- Research Lab: **Health Equity Studio, Population Health Modeler**

The Equity Studio operationalizes the chapter's framework — bring real population data, model SDOH-stratified outcomes.

Slide 20 — How to read Chapter 13 (Equity in Practice)

Book introduces **HEROI** (Health Equity ROI Index).

Platform companion:

- Research Lab: **Health Equity Studio** — HEROI implementation
- [/research-lab/vbc-clinical-quality?tab=risk](#) — VCCI Risk Stratification Lab with SDOH overlay

Read HEROI in the book; calculate one for a real program on the platform.

Slide 21 — How to read Chapter 14 (Operations Pillar)

Book covers revenue cycle under global budgets, HCC coding, workforce, supply chain.

Platform companion:

- [/operations](#) overview
- [/operations/revenue-cycle](#) · [/operations/workforce](#) · [/operations/compliance](#)
- Research Lab: **Transformation Scorecard**

The Transformation Scorecard makes the chapter's operational dimensions self-assessable.

Slide 22 — How to read Chapter 15 (Operations in Practice)

Book has the 30 operational levers and the HCC coding walkthrough.

Platform companion:

- **VBC Readiness Assessment** (operations domain)
- **Evidence Library** for citation-grade references
- [/dashboard/vermont/hospitals](#) — see real Vermont hospitals against these levers

Pick a Vermont hospital; walk it through the 30 levers; use the financial stress test to estimate impact.

Slide 23 — How to read Chapter 16 (Knowledge Transfer)

Book argues transformation requires permanent organizational capability.

Platform companion:

- [/academy](#) — learning paths
- [/academy/personalized-learning](#) — adaptive role-based recommendations
- [/advisory/training](#) — formal team programs

If you're bringing a team to transformation literacy, this chapter is the program design; the Academy is the curriculum.

Slide 24 — How to read Chapter 17 (The Horizon)

Book delivers the Vermont / Oregon / California triangulation.

Platform companion:

- [/vermont-act-68](#) · [/oregon-cco](#) · [/california-calaim](#)
- [/states](#) — the All States Explorer
- [/dashboard](#) — the 50-State Dashboard
- **HTI Dashboard** ([/hti-dashboard](#))

Read the chapter; then sit in front of the HTI Dashboard with the chapter open beside you. The framework becomes data-driven argument.

Slide 25 — How to read Chapter 18 (Political Sustainability)

Book covers coalition strategy, evidence production, institutional anchors.

Platform companion:

- [/vermont-legislative-resources](#) — primary documentary record
- [/policy/feasibility](#) — feasibility studies

Chapter 18 is the most "soft" chapter. Read it with the legislative archive open — the abstract becomes concrete fast.

Slide 26 — How to read Chapter 19 (Portfolio Management)

Book applies PMI discipline to transformation.

Platform companion:

- [/impact-simulation](#) — system-level scenario modeling
- [/transformation-friction-index](#) — the friction map
- **Transformation Scorecard**

Run the Friction Index against your portfolio; use the Scorecard for stage-gate scoring.

Slide 27 — How to read Chapter 20 (AHS Restructuring)

Book is the prescriptive chapter — operational policy advice for state leaders executing Act 68.

Platform companion:

- [/vermont-act-68](#) and [/vermont-act-68/simulator](#)
- [/vermont-rht-program](#) (\$195M RHT)
- [/ahead-model](#)
- [/vermont-designated-agencies](#) — for the behavioral health restructure

Chapter 20 is written at the people on the platform's Vermont pages.

Slide 28 — The "study workflow" — one chapter, one week

Monday: Read the chapter (1.5–2 hrs) **Tuesday:** Open the corresponding pillar page; click every linked tool to skim **Wednesday:** Pick one tool; run a substantive scenario **Thursday:** Take the chapter's tables/checklists and apply them to a real org **Friday:** Bring questions to the AI Analyst — ask cross-references ("how does this connect to Chapter 8?")

20 chapters × 1 week = a 5-month structured curriculum.

Slide 29 — The "research workflow"

For students/researchers writing on a transformation topic:

1. Identify your pillar(s)
2. Read those chapters + the dependency-map chapter
3. Pull citations via the **Evidence Library** (</research-lab/knowledge-workspace?tab=evidence>)
4. Bring your own data into the **Research Workspace**
5. Use the AI Analyst with explicit page/chapter context loaded
6. Save your traversal in </saved>

The platform's Save-to-PDF buttons produce citation-grade exports.

Slide 30 — The "executive briefing workflow"

For executives preparing a board presentation:

1. Run the **HTR Simulator** for your org — get the six-pillar score
2. Identify the lowest-scoring pillar
3. Read that pillar's two chapters
4. Pull the corresponding tool (e.g., **VBC Readiness Assessment**)
5. Use the **Hospital Financial Stress Test** for the economic envelope
6. Use **Save-to-PDF** to assemble a board-ready brief

The platform is structured to support this in under a day.

Slide 31 — The "policy briefing workflow"

For policy professionals preparing testimony or a memo:

1. Locate the relevant policy area in the **Policy Pillar**
2. Read Chapter 4 or 5 of the book for the structural argument
3. Use the **Policy Simulator** to model the proposal
4. Pull comparable evidence from Oregon, California, or other states via </states>
5. Pair with the **Innovation Leaderboard** for cross-state benchmarking
6. Cite via the Evidence Library

Vermont's legislative archive (</vermont-legislative-resources>) is the canonical documentary source for any Vermont-relevant testimony.

Slide 32 — The "Vermont implementer workflow"

For Vermont practitioners executing daily:

1. Bookmark the relevant Vermont program page(s)
2. Open the AI Analyst with that page loaded — pillar context attaches automatically
3. Cross-reference the corresponding chapter of the book when something doesn't add up
4. Track real-time policy changes via The Wire
5. Use the simulators for any decision involving Act 68 thresholds, AHEAD readiness, or VBC contracts

Vermont practitioners get the highest-resolution experience — most pages are written with you in mind.

Slide 33 — Vermont as the connective tissue

The most consistent **book** → **platform** link is Vermont content:

- Every chapter has a Vermont section
- Every Vermont program has a platform page
- Every Vermont program page cites the relevant chapter
- The Wire's Vermont filter keeps the analysis current

Vermont is not "the state we cover most" — it's the **integration mechanism** between the book and the platform.

Slide 34 — Cross-state — Oregon CCO + CalAIM (Chapter 17 worked example)

Book (Chapter 17): the triangulation argument.

Platform (cross-state pages):

- [/oregon-cco](#) — CCO 3.0 (2025–2030)
- [/california-calaim](#) — \$6.7B Medi-Cal transformation
- Compared to Vermont in [/dashboard](#), [/states](#), **HTI Dashboard**

Workflow: Read Chapter 17; open all three state pages in tabs; open the AI Analyst and ask "Compare Vermont's risk-stratification to CalAIM's ECM."

The book argues no single state has solved transformation; the platform lets you triangulate live.

Slide 35 — How the AI Analyst threads everything

The AI Analyst (right sidebar + [/chat](#)) knows:

- The full book (indexed)
- The Sanity content library (reports, webinars, courses)
- Vermont program pages
- Cross-state pages
- Research Lab tool documentation
- The Wire's recent items

When you ask a question on, say, [/vermont-vcci](#), the Analyst attaches that page context **and** the Clinical pillar tag — the answer is grounded in Chapter 10 + the VCCI page + related Wire items.

The Analyst is the cross-reference engine. Use it.

Slide 36 — The "ask the Analyst" pattern

Good Analyst questions:

✔ "How does VCCI's risk stratification compare to CalAIM's ECM tiering?" ✔ "What does Chapter 8 say should precede Act 68's reference-based pricing?" ✔ "Show me the dependency between Technology and Equity, with Vermont examples." ✔ "What's the failure cascade if Vermont's DAs go insolvent?"

Less good questions:

✗ "Summarize the book." (Use the chapter browser.) ✗ "What's healthcare?" (Use a textbook.)

The Analyst is at its best on **cross-chapter**, **cross-pillar**, **cross-state** questions.

Slide 37 — When to use the book vs. the platform

You want to...	Start here
Understand a concept	The book
Run a scenario	The platform
Cite a fact	The Evidence Library on the platform
Get an argument	The book
Track what's changing	The Wire
Compare states	Platform dashboards
Design an APM	APM Design Lab
Score your org	HTR Simulator
Write a memo	Both, then Save-to-PDF

Slide 38 — A worked example — "I run a Vermont hospital under Act 68. Where do I start?"

1. **Read Chapter 4** (Policy) — understand Act 68
2. **Read Chapter 8** (Economics) — understand global budgets
3. **Read Chapter 14** (Operations) — understand revenue cycle under global budgets
4. Open [/vermont-act-68/simulator](#) — model your position
5. Open **Hospital Financial Stress Test** — model your downside
6. Open [/dashboard/vermont/hospitals](#) — see where you sit against peers
7. Open **VBC Readiness Assessment** — score your operations

8. Read **Chapter 20** — the AHS Restructuring roadmap

Three chapters, four tools, one weekend.

Slide 39 — A worked example — "I'm writing testimony on H.R. 1 implications"

1. Read **Chapter 5** — Policy in Practice (Medicaid landscape, H.R. 1)
2. Open **H.R. 1 Cliff Scenario** — model your state's coverage loss
3. Open **Medicaid Work Requirements Calculator** — eligibility math
4. Open **/california-calaim** and **/vermont-medicaid** — contrast
5. Pull citations via the Evidence Library
6. Use the AI Analyst to draft the structural argument

Half a day of platform work, anchored by one chapter.

Slide 40 — A worked example — "I'm a graduate student writing a paper on risk stratification"

1. Read **Chapter 10** (Clinical) — risk-stratification framework
2. Read **Chapter 11** (Clinical in Practice) — operationalization
3. Open the **VCCI Risk Stratification Lab** — explore synthetic patient scenarios
4. Open the **Risk Stratification Methodology Lab** — read the CDPS scoring + SDOH overlay
5. Pull citations from Evidence Library
6. Bring your own institutional data into **Research Workspace**
7. Ask the Analyst: "How does VCCI's tiering compare to the Hochman-Asch tiers used in HCC programs?"

The platform supports student-grade rigor without requiring local statistical infrastructure.

Slide 41 — A worked example — "I'm an executive at a non-Vermont hospital"

1. Read **Chapter 1 + Chapter 2** — get the framework + sequencing
2. Run the **HTR Simulator** for your org
3. Read your two lowest-scoring pillars' chapters
4. Open the corresponding tools
5. Compare to Vermont peers using the Hospital Profiles
6. Use the **APM Design Lab** to design your next contract

Vermont is your benchmark — not because you're in Vermont, but because Vermont is the most measured.

Slide 42 — A worked example — "I'm building a Personalized Learning path"

1. Open **/academy/personalized-learning**
2. Pick your role, your pillar(s), your time budget
3. The platform generates a recommended sequence of chapters + tools
4. As you progress, the Analyst tracks what you've covered
5. Bookmark to **/saved** for later

This is the lowest-effort entry path. Built for first-time users.

Slide 43 — The integration discipline

Every claim on the platform is sourced. Every claim in the book is footnoted. Every chapter has a platform companion section. Every Vermont program has both a book section and a platform page.

The discipline is the integration. Don't trust a "Vermont stat" without a source — and don't accept a source without checking the page.

Slide 44 — Common mistakes (1/3) — Reading without testing

A reader who reads all 20 chapters but never opens the platform tools:

- Knows the framework but cannot apply it
- Knows the dependency map but cannot trace a failure cascade in real numbers
- Knows Vermont as a story, not as a measured system

Use one tool per chapter, minimum.

Slide 45 — Common mistakes (2/3) — Tooling without theory

A user who runs the Simulator, the Stress Test, the APM Lab — but never reads the book:

- Sees results but doesn't know what they mean
- Mistakes correlations in the data for the structural arguments behind them
- Cannot defend a result in a board meeting

Run no tool without reading at least one corresponding chapter.

Slide 46 — Common mistakes (3/3) — Reading Vermont as parochial

Some readers dismiss Vermont as "too small to matter." That is exactly backwards.

Vermont matters because it is:

- The most legislatively complete reform
- The most data-transparent state
- The smallest scale at which the full transformation can be observed
- The benchmark every state-level transformation will be compared against in the 2030s

If you can model Vermont, you can model anywhere.

Slide 47 — The Vermont thread in one slide

Vermont program	Book chapter	Platform page
Act 167 (2022)	4	/vermont-act-167
Act 68 (2025)	4, 8, 20	/vermont-act-68
Blueprint for Health	10	/vermont-blueprint
VCCI	10	/vermont-vcci
SASH	10	/vermont-sash
Designated Agencies	10	/vermont-designated-agencies
8 SDOH Domains	12, 13	/vermont-sdoh
AHEAD Model	8, 17	/ahead-model
RHT Program	4, 20	/vermont-rht-program
Legislative Resources	18	/vermont-legislative-resources

Slide 48 — The Cross-State thread

State	Book treatment	Platform
Vermont	Central case	12+ pages
Oregon	Chapter 17	/oregon-cco
California	Chapter 17	/california-calaim
Maryland	Chapter 8 (HSCRC evidence)	(Wire & analyses)
50-state	Comparative context	/states , /dashboard

The book argues no single state has solved transformation completely. The platform lets you triangulate.

Slide 49 — How the system evolves

- **Book** updates roughly annually (v28 current, v29 in active drafting)
- **Platform** updates continuously — Wire daily, Sanity content weekly, tools monthly
- The **two stay aligned** via a chapter-to-tool map (Appendix G in the book; [/book](#) page on the platform)

When the book moves, the platform moves. When the platform moves, the next book draft incorporates it.

Slide 50 — What's coming in v29

- Expanded SASH Program subsection
- Vermont Designated Agencies subsection
- Expanded VCCI subsection
- Oregon CCO + CalAIM cross-state section
- Updated Appendix G Vermont entries
- Updated Appendix E tool list (24 tools)

The platform already reflects these — the book catches up next.

Slide 51 — A note on currency

The book's argument is **structural** — durable across editions. The platform's data is **current** — updated quarterly at minimum, daily where possible.

When the two disagree:

- For numbers: trust the platform
- For framework: trust the book
- For Vermont policy detail: check the legislative archive

Both can be wrong; the discipline is to triangulate.

Slide 52 — Subscription / access

Tier	Book	Platform
Free	PDF download	Pillar overviews, basic Wire
Subscriber	+ companion materials	Full Research Lab, AI Analyst, Personalized Learning
Student	Academic pricing	Subscriber
Professional	+ advisory office hours	Subscriber + briefings
Advisory	+ custom support	Full custom engagement

Many readers come for the book and stay for the platform; many come for the platform and learn the book matters.

Slide 53 — How instructors use the system

For graduate / professional school adoption:

- The book is the textbook (20 chapters → 15-week semester is natural)
- The platform's tools are the lab exercises
- The Academy's learning tracks become weekly problem sets
- The AI Analyst is the TA on call

Instructor support package available via Advisory.

Slide 54 — How advisory clients use the system

Engagement model:

1. Joint diagnostic — HTR scores the client on the six-pillar framework
2. Find the lowest-scoring pillar
3. The book chapter becomes the briefing material for the client's leadership
4. The platform's tools become the analysis instruments
5. HTR consultants embed alongside the client team
6. Outputs: a transformation roadmap + a client-specific scorecard

The book is the syllabus; the platform is the lab; the consultant is the guide.

Slide 55 — Why the integration matters

Most healthcare reform writing is:

- Book without data (dated within a year)
- Data without framework (correlations without causation)
- Tool without theory (a calculator with no opinion)

The HTR system is built so each piece compensates for the others' weaknesses:

- Book provides the **framework**
- Platform provides the **currency**
- AI Analyst provides the **connections**

The integration is the product. Neither piece is the product alone.

Slide 56 — The user's promise

If you commit to the system:

- You will learn to read state-level transformation as **system behavior**, not headlines
- You will model real policy/payment/clinical scenarios on real data
- You will have a defensible structural framework to argue from
- You will know where Vermont sits in the national picture — and what that means for your work
- You will not be the person in the meeting who says "but what about California?" — you'll already have the data

Slide 57 — How to start — first 90 minutes

1. **Slide 1 of the platform** — open [/book](#) and read the four reader profiles. Pick yours.
2. **Open the book** — read the Preface and Introduction (30 min)
3. **Open [/about/framework](#)** — interact with the dependency map (10 min)
4. **Open the HTR Simulator** — score yourself or Vermont (30 min)
5. **Open the AI Analyst** — ask one question (20 min)

By end of day 1, you should understand the system's shape.

Slide 58 — How to start — first week

1. Day 1 — Preface, Introduction, Chapter 1, the platform tour
2. Day 2 — Chapter 2, OneCare autopsy, the Impact Simulation
3. Day 3 — Your primary pillar chapter (e.g., Chapter 8 if you're an executive)
4. Day 4 — The corresponding pillar page + one Research Lab tool
5. Day 5 — Vermont content ([/vermont-act-68](#), [/vermont-vcci](#))
6. Day 6 — Catch up + The Wire
7. Day 7 — Bring a real question to the AI Analyst

Slide 59 — How to start — first month

By end of month 1, you should:

- Have read the book's foundational chapters (Preface, Intro, 1–3) plus your two primary pillar chapters
- Have run scenarios in ≥3 Research Lab tools
- Have a [/saved](#) library of ~10 items
- Have brought ≥5 substantive questions to the AI Analyst
- Have a working mental model of the Vermont thread

Slide 60 — How to start — first quarter

By end of quarter 1, you should:

- Have read all 20 chapters (~1.5/week)
- Have used ≥10 Research Lab tools
- Be able to cite Vermont policy detail from the legislative archive
- Have a defensible six-pillar score for an organization you care about
- Be able to predict the failure cascade for a given pillar gap

This is **mastery-level** engagement. Most readers reach it in 4–6 months part-time.

Slide 61 — Pitfalls to avoid

- Reading linearly without using the platform — don't

- Tool-hopping without reading the chapters — don't
- Treating Vermont as anecdotal — don't
- Dismissing the dependency map as theoretical — it's predictive
- Using the AI Analyst as a search engine — use it for cross-references
- Skipping Appendix D (the VBC Contract Review Checklist) — it's the most-used artifact in the system

Slide 62 — The community of practice

You're not alone in this:

- [/connect](#) — the member directory
- [/connect/directory](#) — find practitioners by pillar and state
- [/community](#) — discussion threads
- [/academy/faculty](#) — contributing experts
- HTR Advisory — direct expert engagement

The system has a network effect — more practitioners on it means more value for everyone.

Slide 63 — Beyond the system

When the system isn't enough:

- For primary research data — VHCURES (Vermont), HCUP (national), MEPS (national)
- For state-by-state Medicaid detail — KFF, MACPAC
- For peer-reviewed evaluations — Health Affairs, Health Services Research, JAMA Health Forum
- For Maryland's global-budget evidence — HSCRC publications
- For federal policy currency — CMMI announcements, KFF, CBO

The book and platform integrate these — but they don't replace them.

Slide 64 — Final framing

The HTR system is one answer to a hard question:

How do you train a generation of healthcare leaders to do transformation, given that transformation has no precedent at scale and no textbook?

The answer here:

- A book grounded in the most complete state-level case
- A platform that keeps the analysis live
- A framework that lets readers test what they read

If that's the question you're working on, you've found the right system.

Slide 65 — Get started

- Book: download from [/book](#) or print via Advisory
- Platform: request beta access via [/beta](#)
- Subscription: [/pricing](#)
- Advisory: advisory@healthtransformationreview.com
- AI Analyst: open on any page, or [/chat](#)

Thank you.

End of Combined Deck (65 slides).